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Correct Answer

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Practice Exam - 1

Question 76 of 117



A 61-year-old female with a 5-year history of chronic ischemic cardiomyopathy and left ventricular ejection fraction of 25% is hospitalized for decompensated heart failure. She has generally done well on a daily oral diuretic dose of furosemide 80 mg, but was recently discharged from her first heart failure hospitalization 3 weeks prior. Due to an increase in serum creatinine from 1.0 to 1.5 mg/dL after in-hospital diuresis and achievement of euvoemia, she was discharged on a decreased maintenance oral diuretic dose of furosemide 40 mg daily. She now presents with dyspnea on exertion, mild leg edema, abdominal bloating and an 8 lb. weight gain. During her current admission, she undergoes diuresis with intravenous loop diuretics and optimization of heart failure guideline recommended medical therapy - including optimization of beta blocker, ARNI, MRA, and SGLT2 inhibitor therapy as tolerated

Which of the following is the most effective guideline recommended intervention to reduce the patient's risk of 30-day readmission?

- A. Transition from oral furosemide to oral torsemide
-  B. Referral for nephrology evaluation
- C. Reassessment of renal function and electrolytes within 1-2 weeks
-  D. Follow-up appointment within 7 days of discharge

Commentary

References

Testing Point: The Board-certified AHFTC specialist should recognize optimal management strategies at time of hospital discharge to prevent rehospitalization.

Correct Answer: D. Follow-up appointment within 7 days of discharge

Rationale: According to the 2022 AHA/ACC/HFSA heart failure guidelines, it is a Class II recommendation for a post-discharge follow-up appointment within 14 days of hospital discharge. Given the complexity of the heart failure medication regimen, and changes that occur once a patient is home in lifestyle and diet, the first post-discharge visit affords an important opportunity to perform medication reconciliation, patient education and treatment optimization. Scheduling an appointment prior to patient discharge may improve

rates of appointment completion. Close follow-up with a heart failure specific disease management program when available is recommended.

Answer A: Transitioning from oral furosemide to oral torsemide may be considered in patients on high (>80 mg/day) doses of furosemide, with demonstrated lack of response or decreased bioavailability. However, in this patient, the dose of furosemide was recently reduced, and there is no existing evidence that a switch to torsemide at this juncture would reduce readmission. The recent TRANSFORM trial showed no difference in rates of rehospitalization if patients were discharged on torsemide vs furosemide, further diminishing the likelihood that this intervention will make a difference.

Answer B: While chronic kidney disease may be playing a role in this patient's disease trajectory, this is, more a longer term consideration. This answer may be chosen given the patient's prior worsening renal function, however is less likely to impact the patient's short-term risk of readmission. In fact, based on existing diuretic clinical trial and registry data, worsening renal function may be expected with effective diuresis, without impact on longer term outcomes.

Answer C: Close monitoring of renal function and serum potassium is encouraged given the nuances of diuretic and other heart failure guideline directed medical therapies, however usually prompted by a specific recent change in medication or dosing. This intervention should be considered as part of the more comprehensive post-discharge follow up appointment and has no demonstrated benefit to reduce hospitalization.

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